

August 12, 2026

South Carolina Medicaid (Healthy Connections / SCDHHS)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Renato Dare (MRN MUSC8182824), Claim MUSC-FFD5-3

Patient	Renato Dare (DOB 1976-06-14)
MRN	MUSC8182824
Member ID / Group	SCD346795729 / GRP-86928
Claim number	MUSC-FFD5-3
Date of service	2026-06-05
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$495.45
Denial code	CARC 197 / RARC N54 — Precertification/authorization/notification/pre-treatment absent.
Appeal deadline	2026-08-10

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this formal first-level appeal on behalf of patient Renato Dare (MRN: MUSC5811662, Member ID: SCD346795729) regarding the denial of Claim ID MUSC-FFD5-3 for services rendered on June 5, 2026, by Robert K. Lindqvist, MD (NPI 1314059687) at MUSC Health Rutledge Tower Ambulatory Care. The claim was submitted on June 11, 2026, for CPT code 99214 (office/outpatient visit, established patient, moderate complexity), billed at \$495.45. South Carolina Medicaid (Healthy Connections / SCDHHS) denied the claim in full on July 11, 2026, citing CARC 197 — "Precertification/authorization/notification/pre-treatment absent" — and RARC N54 — "Claim information is inconsistent with pre-certified/authorized services" — with the payer remark: "No prior authorization on file for the date of service billed." MUSC Health respectfully disputes this denial and requests full reimbursement of the billed amount.

CLINICAL BACKGROUND

Mr. Dare is a 50-year-old male with an extensive and active chronic disease burden managed longitudinally at MUSC Health. His documented active conditions include type 2 diabetes mellitus with peripheral neuropathy, prediabetes, metabolic syndrome, hypertriglyceridemia, hyperglycemia, and anemia. He has been maintained on extended-release metformin since November 2007. He also carries a history of recurrent acute upper respiratory infections, including prior episodes of acute viral pharyngitis and viral sinusitis, each coded under J06.9. The June 5, 2026 encounter was billed under primary diagnosis J06.9 (acute upper respiratory infection, unspecified) with a secondary diagnosis of R69, consistent with his concurrent metabolic conditions. Given the complexity of Mr. Dare's comorbid profile, the selection of a moderate-complexity established-patient visit level (99214) is clinically appropriate and well supported by the documented conditions requiring evaluation and management.

BASIS FOR APPEAL

South Carolina Medicaid policy governing outpatient evaluation and management services for established patients does not universally require prior authorization for routine office visits, particularly when rendered by an established treating provider in an outpatient setting. CPT code 99214, a standard established-patient office visit, is a widely recognized, non-invasive evaluation and management service. Prior authorization requirements under Healthy Connections are generally reserved for specific high-cost procedures, specialty services, or durable medical equipment as enumerated in the program's covered services guidelines — not for routine ambulatory E&M; visits of this nature. MUSC Health has no record of a prior authorization requirement having been communicated to the facility or the rendering provider for this category of service on this date of service. To the extent that South Carolina Medicaid maintains that authorization was required, MUSC Health respectfully requests that the payer identify the specific provision of the Healthy Connections covered services policy that mandates prior authorization for an established-patient office visit at this complexity level, rendered in an on-campus outpatient hospital setting (Place of Service 22). If no such requirement exists or if the denial was issued in error, the claim should be reprocessed and paid at the applicable Medicaid fee schedule rate. Additionally, MUSC Health notes that the allowed amount on the claim was calculated at \$297.06, indicating the payer's own adjudication system recognized a payable amount prior to applying the authorization denial — further supporting the position that the underlying service was covered and appropriately coded.

REQUESTED ACTION

MUSC Health respectfully requests that South Carolina Medicaid (Healthy Connections / SCDHHS) overturn the denial of Claim ID MUSC-FFD5-3 and reprocess the claim for payment at the previously calculated allowed amount of \$297.06, or at the applicable Medicaid fee schedule rate, whichever is appropriate. Should the payer require additional clinical documentation, operative notes, or provider attestation to support this appeal, MUSC Health will provide those materials promptly upon request. This appeal is submitted within the 30-day appeal window, prior to the August 10, 2026 deadline, and is directed to the Division of Appeals and Hearings in accordance with the payer's stated appeal rights.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record